

August 20, 2026

South Carolina Medicaid (Healthy Connections / SCDHHS)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Regena Moore (MRN MUSC8962062), Claim MUSC-B4EA-1

Patient	Regena Moore (DOB 1964-12-08)
MRN	MUSC8962062
Member ID / Group	SCD742523430 / GRP-85832
Claim number	MUSC-B4EA-1
Date of service	2026-06-30
Procedure billed	CPT 74177 — CT abdomen and pelvis with contrast
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$2,674.63
Denial code	CARC 11 / RARC M64 — The diagnosis is inconsistent with the procedure.
Appeal deadline	2026-08-26

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this formal first-level appeal on behalf of patient Regena Moore (MRN: MUSC8962062, Member ID: SCD742523430) regarding the denial of Claim MUSC-B4EA-1 for services rendered on June 30, 2026, at MUSC Health University Medical Center. The claim was denied on July 27, 2026, under CARC 11 ("The diagnosis is inconsistent with the procedure") and RARC M64 ("Missing/incomplete/invalid other diagnosis"), with the payer remark stating: "The diagnosis code reported does not support the procedure billed." The total denied amount is \$2,674.63. MUSC Health respectfully requests that South Carolina Medicaid (Healthy Connections / SCDHHS) reverse this denial and reprocess the claim for appropriate payment.

CLINICAL BACKGROUND

Ms. Moore is a 61-year-old female patient with active diagnoses including anemia (D64.9) and prediabetes (E11.9), both of longstanding duration. On June 30, 2026, she presented as an outpatient at MUSC Health University Medical Center, where rendering provider Amara J. Boone, NP (NPI 1203948576) ordered CPT 74177, a CT scan of the abdomen and pelvis with contrast. The claim was submitted on July 7, 2026, with J06.9 (Acute upper respiratory infection, unspecified) listed as both the primary and secondary diagnosis code. The procedure was performed in an on-campus outpatient hospital setting under revenue code 0510.

BASIS FOR APPEAL

The denial is well-founded with respect to the diagnosis code as submitted, and MUSC Health acknowledges that J06.9 — Acute upper respiratory infection, unspecified — does not, on its face, provide clinical justification for a CT abdomen and pelvis with contrast. However, the denial reflects a coding deficiency rather than a lack of medical necessity for the procedure itself. The claim was submitted with an incomplete and clinically inaccurate primary diagnosis code that does not reflect the full picture of Ms. Moore's active conditions or the clinical indication that prompted the imaging order.

Ms. Moore carries active diagnoses of anemia (D64.9) and prediabetes (E11.9), both of which are documented in her longitudinal record. Additionally, her history includes a prior tubal pregnancy and a first-trimester miscarriage, conditions that are part of her gynecologic and abdominal history and may bear clinical relevance to abdominal and pelvic imaging. The presence of anemia, in particular, is a condition that can prompt evaluation of the abdomen and pelvis to investigate underlying etiology, and it represents a clinically supportable indication for the procedure performed. The submission of J06.9 as the sole diagnosis appears to be a coding error in which the appropriate diagnosis or diagnoses reflecting the true clinical indication were not captured on the claim.

Under correct coding principles consistent with ICD-10-CM guidance, the diagnosis reported on a claim must reflect the condition that prompted the service, not an unrelated or incidental condition. The failure to report the correct diagnosis code constitutes a correctable billing error, not a determination that the service was medically unnecessary. South Carolina Medicaid's appeal and claims correction processes recognize that coding errors are subject to review and correction upon submission of supporting documentation.

REQUESTED ACTION

MUSC Health respectfully requests that South Carolina Medicaid (Healthy Connections / SCDHHS) review the complete medical record for the June 30, 2026, encounter, consider the active diagnoses documented in Ms. Moore's record — including anemia (D64.9) — as the clinically appropriate basis for the CT abdomen and pelvis with contrast, and either reprocess the claim with the corrected diagnosis coding or provide guidance to allow MUSC Health to submit a corrected claim. We further request that the allowed amount of \$1,578.45 already established by the payer be honored upon reprocessing. All supporting clinical documentation is available upon request and will be submitted promptly to facilitate timely resolution prior to the August 26, 2026, appeal deadline.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

• Remittance advice / explanation of payment

- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record